

End-Stage Scarring Alopecia

End-stage disease is characterized by nonspecific scarring alopecia, leading to the use of several clinical terms that likely describe the same entity: **lichen planopilaris**, **folliculitis decalvans et atrophicus**, **lichen spinulosus at folliculitis decalvans**, and **Graham-Little syndrome**.

Pseudopelade of Brocq is a rare clinical syndrome of scarring alopecia and fibrosis lacking distinct pathologic features. It is generally accepted as the end stage of follicular fibrosis caused by primary inflammatory dermatoses such as: - **Lichen planus** - **Lupus erythematosus** - **Pustular-scarring folliculitis** - Fungal infections (e.g., favus) - **Scleroderma** - **Sarcoidosis**

Mucosal Lichen Planus

Lichen planus can affect mucosal surfaces including the **mouth, vagina, esophagus, conjunctiva, urethra, anus, nose, and larynx**. Its prevalence is estimated at approximately **1% of the adult population**.

- **Oral involvement** occurs in **60–70%** of patients with lichen planus.
- In **20–30%** of cases, oral lesions may be the only manifestation.

Clinical Forms of Oral Lichen Planus

Different types have been described: - **Reticular** (most common overall) - **Plaque-like** - **Atrophic** - **Papular** - **Erosive-ulcerative** (more symptomatic; commonly seen in clinical practice) - **Bullous**

The **reticular form** is most frequent, but **erosive forms predominate in oral medicine clinics** due to their chronicity and symptom burden. Erosive lichen planus is more common in the elderly and typically causes **painful, burning sensations**.

Common Sites of Oral Involvement

- Buccal mucosa
- Gingiva
- Glossal (tongue) mucosa

Less commonly affected sites include: - Palate - Floor of the mouth - Retromolar pads - Lips

Gingival involvement may present as: - Gingival stomatitis - Desquamative gingivitis (seen in 8–10% of oral lichen planus cases as the sole manifestation)

Oral Lichenoid Reactions

Oral lichenoid reactions may occur on the **buccal mucosa adjacent to amalgam dental fillings**. Patch testing often reveals **positive reactions to mercury, gold**, and other metals.

Due to the **chronic course, pain, and discomfort**, patients with oral lichen planus are at increased risk for depression, and **psychological support** should be offered.

Lichen Planus in HIV Infection

Bilateral **reticular keratotic or atrophic changes** of the buccal mucosa and **lichenoid atrophic patches** on the dorsal tongue have been reported in patients with **HIV infection**, often following **zidovudine or ketoconazole** use. Histologic findings include: - Liquefaction degeneration of basal keratinocytes - Epithelial atrophy - Lichenoid lymphocytic infiltrate

Genital and Anal Involvement

Male Genitalia

- Affected in **25%** of cases
- Most commonly involves the **glans penis**

- Lesions are often **annular**

Female Genitalia

Manifestations include: - Patches of **leukoplakia** or **erythroplakia** - **Erosions** - **Generalized desquamative vaginitis**

Complications may include: - **Vaginal adhesions** - **Labial agglutination**

Anal Lesions

Present with: - **Leukokeratosis** - **Hyperkeratosis** - **Fissuring** - **Erosions**

Vulvovaginal-Gingival Syndrome

A distinct variant of mucosal lichen planus involving **vulvar and gingival tissues** has been described.

Key Features:

- **Gingiva and tongue:** erythema, erosions, occasionally white reticulated plaques
- **Vulva and vagina:** desquamation, erosions, burning pain, dyspareunia, vaginal discharge

This syndrome mirrors oral findings and is associated with significant discomfort.

Ocular Involvement

Conjunctival lichen planus may present as **cicatricial conjunctivitis**.

Histopathology shows: - Irregular thickening - Reduplication of the basement membrane